LIBBY — PATIENT/CAREGIVER TRACK

gist-sdh-multifocal-resected-m1-k4n8

Plain-language summary for patient and caregiver

Generated 2026-08-21

What this page is

Libby is a research tool that reads the published evidence for one person's cancer and lays out the options in order. This page is the plain-language version of a longer report written for clinicians. It is meant to help you have a better conversation with your own oncologist, not to replace it.

The short version of your case is unusual, and it is good news as far as it goes: the surgery removed all of the visible cancer, so there is nothing to treat right now. The main message below is that careful watching, not another drug, is the right plan at this point. There is also a useful test worth doing now, and a menu of options to keep in your back pocket in case the cancer ever comes back.

What we know about your cancer

You have a gastrointestinal stromal tumor, or GIST, which started in the stomach. Most GISTs are driven by a change in a gene called KIT or PDGFRA. Yours is not. It belongs to a rarer group called SDH-deficient GIST, driven instead by changes in a gene called SDHA. This distinction matters a lot, because the usual GIST treatments were built around the KIT and PDGFRA type and do not work the same way here.

The cancer was in more than one spot in the stomach, and there was one deposit a little further down the digestive tract. Surgery removed all of it, with clean margins, and the lymph nodes that were checked were clear. So at the moment there is no cancer that can be seen on scans. The medical shorthand for that is NED, "no evidence of disease."

Two things are worth knowing about this subtype. First, it tends to grow slowly, often over years rather than months. Second, even after a clean removal, it comes back fairly often, which is why a monitoring plan matters. Slow-growing is not the same as gone.

What you told us matters most

You did not give us a detailed list of preferences, so we used sensible defaults and want to be upfront about that. We assumed you would lean slightly toward treatments that work, while still caring about side effects, and that you would be open to clinical trials, since the most promising targeted options for this rare subtype mostly live in trials. If any of that is wrong, it is worth telling your care team, because a couple of the choices below hinge on how far you are willing to travel and how you feel about living with regular scans.

The plan right now: careful monitoring

With no visible cancer and a slow-growing subtype, the recommendation is to watch closely rather than start another drug. Here is the reasoning, plainly: no added treatment has been shown to lower the chance of this subtype coming back. The studies that justify giving a drug after GIST surgery were done in the KIT-driven type and did not include people with your subtype. And the one standard GIST pill that gets reached for by reflex barely works here, which we come back to at the bottom. So starting a drug now would mean signing up for months or years of side effects in exchange for very little, while also using up your eligibility for trials you might want later.

What monitoring actually looks like:

- Regular scans, roughly every three to six months for the first couple of years, then spaced further apart if things stay quiet.
- A baseline check for a related kind of tumor (called paraganglioma or pheochromocytoma) that people with SDH gene changes have a higher chance of developing. This is usually a blood or urine test plus an imaging scan. It matters for a practical safety reason too: one of these tumors, if undetected, can cause a dangerous spike in blood pressure during anesthesia, so it is worth knowing about before any future surgery.

The honest catch with monitoring is that it only protects you if it actually happens on schedule, and living with regular scans carries a real anxiety cost that does not show up on any side-effect chart. If the waiting weighs on you, say so. That is a legitimate part of the decision.

First steps worth doing now

Two things are worth setting in motion now, while you feel well. Neither involves any cancer drug, and neither has side effects beyond a blood draw.

A repeat test on the stored tumor tissue

There is a small puzzle in your results. The gene findings point clearly to the SDH-deficient subtype, but one of the protein stains (called SDHB) came back looking normal, which usually is not the case when those gene changes are present. That mismatch is worth sorting out, because the answer decides which future options are open to you.

The test is a re-stain of tissue already stored from your surgery (SDHB and SDHA staining, with a proper control), along with a blood test of your own genes and a confirmation of a few findings that so far have only shown up in blood-based testing. It is non-toxic, it uses tissue you already gave, and it usually takes somewhere between a couple of weeks and a month to come back.

Why it matters: nearly every clinical trial for this subtype uses that SDHB stain as the entry ticket. If the re-stain confirms the diagnosis, the whole menu below stays open. If, after a clean repeat, the subtype is not confirmed, then the trial-based options come off the table, and the next step would be a different specialized test. We come back to what a non-confirming result means in the menu section.

Genetic counseling

This subtype can run in families. The gene change behind your cancer may be one you were born with rather than one the tumor picked up, and the blood test above will help tell which. Either way, meeting with a genetic counselor is recommended. They can talk through what the result means for relatives and for your own long-term screening. One incidental finding in your results (a change in a gene called POLE) most likely turns out to be harmless, and a counselor can close that loop so it does not get over-read.

There is also a national natural-history study for rare tumors like yours that you can join even with no visible cancer. It is observational, meaning it does not involve any experimental drug. Joining gives you access to a specialist center, banks tissue that could matter if a trial slot opens later, and folds neatly into the genetics conversation. The main thing to weigh is travel, since it usually involves one in-person visit.

If the cancer ever comes back: the menu of options

None of the following is for now. They are contingencies to discuss only if and when the cancer returns and can be measured on a scan.

These options depend on the re-stain confirming the SDH-deficient subtype. If that test confirms it, the options below are on the table. If it does not confirm it, even after a clean repeat, these come off the table, because Libby only ranks treatments aimed at your specific tumor feature. A non-confirming result would mean there are no further options ranked on this page, and standard care for your cancer would become a separate conversation with your treating team. That conversation is theirs to have with you; it is outside what this page covers.

A general note on the newer drugs here. Several rest on small, early studies, sometimes only a couple of dozen people, sometimes fewer. The results so far are encouraging, but encouraging results from small numbers are less certain than results from large, long-running trials. We have tried to be clear about which is which.

Option 1 — Sunitinib (a daily pill)

Of everything on this list, sunitinib has the most track record in your subtype. In a study of people with SDH-deficient GIST, about 18 out of every 100 had their tumors shrink on sunitinib, compared with about 2 out of every 100 on the standard GIST pill. That 18 is honest but modest, and the data come from looking back at past cases rather than a planned head-to-head trial, so treat it as a real signal rather than a precise promise.

It is an approved drug, taken by mouth, and after two decades of use its side effects are well understood: fatigue, diarrhea, sore or discolored hands and feet, and raised blood pressure are the common ones, and there are well-worn ways to manage them. Blood pressure and skin would be watched from the start. This fits your stated lean toward predictable, manageable treatment, though it is not a trial, so it sits alongside the trial options below rather than ahead of them.

Option 2 — Temozolomide (a pill), best through a trial

This option carries caveats. Here is the tradeoff. In a very small study, 2 of 5 people with this subtype had their tumors shrink on temozolomide, which is a high proportion but rests on just five people, so it is more of a promising hint than solid proof.

There is also a catch that makes an extra test worth doing. Temozolomide tends to help only when a marker called MGMT is "switched off" in the tumor, which happens in roughly a quarter of these cancers. A simple test can tell you whether yours is in that group. Without it, you might take on the drug's side effects (low blood counts, fatigue, nausea) for little benefit. So the sensible move is to order the MGMT test ahead of time, so the answer is ready if this ever becomes live. The preferred way to use temozolomide here is through a clinical trial.

Option 3 — Olverembatinib (a pill), trial only

This option carries caveats. This is one of the newer drugs, and the early results are genuinely encouraging: in a study of 26 people with your subtype, about 23 out of 100 had their tumors shrink, and on average the cancer stayed in check for around two years before growing again. It also had the gentlest side-effect profile of the newer options, with anemia and liver-test changes being the main issues.

The caveats are real, though. This was a single early study with no comparison group, so the numbers could look different in a larger, more rigorous trial. The way the drug works is also a bit indirect for a tumor like yours, so its benefit is borrowed from related biology rather than proven head-on. And the trial is based in China, so access is a separate hurdle from whether you would qualify. One eligibility wrinkle: it requires that SDHB stain to show loss, which is exactly the point your re-stain needs to settle.

Option 4 — Rogaratinib (a pill), trial only

This option carries caveats. On paper this has the highest response numbers of anything on the list: in a study of 24 people, about 42 out of 100 had their tumors shrink, and the cancer stayed in check for around two and a half years on average. That is the strongest efficacy signal reported in this subtype so far.

Two things hold it back, and they are worth being clear-eyed about. First, the detailed side-effect breakdown for this drug is not publicly available, so we genuinely cannot tell you how often the serious side effects happened. That is a real gap, not a formality. Second, the study was small and had no comparison group, and the trial has since closed to new patients, with the company stepping back from the program. So even if it looks like the best option by the numbers, getting access to it is the wall, and the related drug in the next option is the more realistic way to chase the same idea.

Option 5 — Belzutifan (a pill), off-label or through a trial

This option carries caveats. Belzutifan is interesting because it aims at the underlying driver of this cancer more directly than the others. But there is no published result yet in GIST. The only supporting evidence comes from a cousin disease with the same gene problem, where about 26 out of 100 people responded. Whether that carries over to GIST is unknown, and there is a cautionary precedent: another drug aimed at similar biology was tried in this subtype and helped no one. So the logic is appealing and the proof is not there yet.

One practical point in its favor: it is already approved for other cancers, so in principle it can be used without waiting for a trial slot. Two side effects to flag are fatigue and anemia, and the anemia could be worse for you specifically because the type of stomach surgery you had can make it harder to absorb iron and vitamin B12.

Option 6 — Pemigatinib (a pill), through a trial

This option carries caveats. Pemigatinib works the same way as rogaratinib (Option 4) but through a door that is actually open: there is a trial recruiting people with your exact subtype, based in the US. The honest limitation is that there is no result yet for this specific drug in this cancer. The case for it rests entirely on the rogaratinib numbers and shared laboratory findings. For someone open to trials, it is the more practical way to test the same approach, even though rogaratinib carries the bigger published number. Side effects to know about include high phosphate levels, mouth sores, and a particular eye problem that needs monitoring.

Option 7 — Regorafenib (a pill), a later-line option

This option carries caveats. Regorafenib is an approved GIST drug usually used after sunitinib. The trouble is there is no data on how well it works specifically in your subtype. Its approval comes from the KIT-driven type, and its use here is an educated extrapolation rather than a tested result. Treatment guidelines list it as the next rung after sunitinib, but with that gap stated plainly. Side effects include sore hands and feet, raised blood pressure, and diarrhea, which can be significant.

Option 8 — A Dutch matched-drug program (DRUP)

This option carries caveats, and it is a late, marginal one. This is not one of the SDH-targeted choices above, and unlike the rest of this menu it does not depend on the re-stain. It is a different kind of route worth knowing exists, but it is far down the list for good reasons.

DRUP is a program in the Netherlands that tries to match patients to drugs already approved for other cancers, based on molecular changes in their tumor. Two changes turned up in your blood-based (ctDNA) testing, and each one has an existing drug aimed at it. On paper, that is what DRUP could offer: a way to access one of those already-approved drugs.

Here is why it is a marginal option rather than a real recommendation. First, those two changes have only shown up in blood so far, not in the tumor tissue itself, so they would need to be confirmed in tissue before anyone could act on them, and one of them is an unusual version that may not respond to its matching drug the way the textbook versions do. Second, DRUP does not report how well any single drug-and-change pair works. It only publishes a pooled figure across all its patients and all its drugs together, so there is no number that tells you the odds for your specific situation. It is an access door, not evidence that these drugs would help you. Third, the program requires that the cancer has come back and grown despite standard treatment, so it is a late-line route, not an early one. And it is based in the Netherlands, so reaching it is a separate problem from qualifying for it.

One more thing to be straight about: this option was added after the expert-panel review of your case, from a later search of the trial registry. The panel that weighed the other options did not look at this one. It is listed so the door is visible, not because it earned a place alongside the choices above.

One option that was considered and set aside

Imatinib (the standard GIST pill) — not recommended here

This option was considered but not recommended. It is on the page because Libby shows what was weighed and rejected, not only what was kept. Imatinib is the first drug most people think of for GIST, and for the common type it works well. For your subtype it does not. In the relevant study, only about 2 of every 100 people responded. The major trials that established imatinib after GIST surgery did not include this subtype at all.

Taking it would mean roughly three years of side effects (puffiness around the eyes, fatigue, low blood counts, rash, and regular monitoring) in exchange for almost nothing, and it would use up the trial eligibility you would rather keep. Both major treatment guidelines specifically advise against it for this subtype. We flag it here so the reflex "GIST means imatinib" reach is on the record and does not get quietly re-made later.

Questions to ask your oncologist

- How often will I have scans, and what kind (CT or MRI), and what would trigger a change in the schedule?
- Can we get the SDHB and SDHA re-stain ordered now, and what does it mean for my options if the diagnosis is not confirmed after a clean repeat?
- Should I have the blood test for an inherited gene change, and can you refer me to a genetic counselor for me and my family?
- What is the plan for the baseline paraganglioma screening, and what should the anesthesia team know before

any future operation?
- If the cancer comes back, would you start with sunitinib, and how would we watch my blood pressure and skin in the early weeks?
- Can we test the tumor for the MGMT marker ahead of time, so we know whether temozolomide is worth considering if it is ever needed?
- For the trial-based options, which ones have sites I could realistically reach, and would it help to connect with a specialist center now rather than later?
- How would we decide between sticking with the approved pill and trying a newer trial drug if a recurrence happens?
- The two blood-only changes that the Dutch matched-drug program keys on: is it worth confirming them in tumor tissue, and would you consider that route realistic given it is overseas and later-line?

Sources

Recommendations on this page draw on the following references, listed for your care team.

- Published studies (PubMed IDs): 27011036, 29413424, 30383140, 39927693, 35546442, 23023976, 21173220, 32424176, 17046465, 34426440, 36198483, 41184234, 42191879, 31666694, 35324464, 15652751, 23177515, 40045030, 31570881, 35046062, 19303137.
- Clinical trials (ClinicalTrials.gov IDs): NCT03739827, NCT05661643, NCT06640361, NCT04595747, NCT04924075, NCT07434843, NCT02925234.